Structured Emergency Assessment and Perfusion Change

NURSING · nursing-foundations · BoardPrep lesson v1

Official examination scope

PRC Resolution No. 10 s. 2025 and the August 2026 NLE program define examination scope, not topic weights. This lesson teaches a bounded initial emergency assessment and shock recognition foundation from the references below. Open RN and U.S. professional materials support general educational principles only; apply Philippine law, scope of practice, current clinical orders, referral pathways, and facility policy. This study material is not a clinical directive or proof of complete NP coverage.

Learning objectives

- Use ABCDE as an initial structure for time-sensitive assessment while addressing an immediate
- life threat and following local escalation protocol.
- Recognize a worsening perfusion pattern from multiple cues and communicate change, response,
- and need for help.
- Reassess after an intervention and avoid treating a mnemonic as a diagnosis or treatment
- order.

1. Organize an initial assessment around immediate threats

ABCDE organizes a first assessment: airway, breathing, circulation, disability, and exposure/environment. Address an immediately life-threatening problem as it is found within training and local protocol, call for support early, and reassess before moving on because the patient can change. The sequence is a cognitive aid, not a reason to ignore scene safety, a patient's specific presentation, or a required local pathway. SAMPLE history can add context after immediate threats are considered. This lesson does not specify medication, procedure, pediatric variation, transfer criteria, or numeric threshold; use the applicable Philippine clinical standard and team instruction.

Sources for this section

Section reference: Basic Emergency Care: Approach to the Acutely Ill and Injured — World Health Organization and International Committee of the Red Cross — https://www.who.int/publications-detail-redirect/9789241513081
Section reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

2. Recognize a concerning perfusion trend

Shock is a time-critical failure of adequate tissue perfusion; no single finding proves its cause. New confusion, cool or clammy skin, weak pulses, and declining urine output form a concerning pattern when considered with history and trends. Assess the person, verify observations when safe, seek urgent help through the local emergency response pathway, communicate the pattern and changes, and reassess while the authorized team directs care. Do not wait for a textbook threshold, attribute the pattern to one shock type without evidence, or start a fluid or medication regimen from this educational summary. WHO Basic Emergency Care supports systematic recognition and response; local protocols determine treatment.

Sources for this section

Section reference: Basic Emergency Care: Approach to the Acutely Ill and Injured — World Health Organization and International Committee of the Red Cross — https://www.who.int/publications-detail-redirect/9789241513081

Apply an initial ABCDE structure

Question: An acutely ill patient arrives while routine tasks are pending.
1. Identify immediate airway and breathing threats first while checking scene and team safety.
2. Continue through circulation, disability, and exposure/environment, responding to a life threat as found under local protocol.
3. Reassess after action and give a concise handoff with findings and response.
Answer: ABCDE structures urgent assessment; routine work waits when an immediate threat is present.
Check: The sequence does not replace judgment, authorized treatment, or local escalation instructions.

Recognize a concerning perfusion pattern

Question: A patient develops new confusion, cool clammy skin, weak pulses, and decreasing urine output.
1. Compare with baseline and verify the timing and trend of observations.
2. Treat the cluster as possible worsening perfusion and request urgent assistance.
3. Communicate observed cues and response, then continue reassessment as directed.
Answer: The pattern warrants urgent assessment and escalation; it does not identify a cause or prescribe treatment.
Check: No numeric cutoff or fluid order is inferred.

Common misconceptions

- ABCDE is a prompt for structured reassessment, not a diagnosis or treatment protocol.
- One sign does not identify shock type; trends and the whole clinical context matter.
- A numeric cutoff is not required before escalating a concerning deterioration pattern.
- Treatment choices require current local protocols and authorized clinical direction.

Practice patterns

- In an emergency question, prioritize immediate life threats, use a structured first
- assessment, and reassess.
- For possible shock, recognize a multi-cue perfusion pattern and escalate without inventing a
- cause or regimen.

Official scope references

Official scope: Enhanced Tables of Specifications for the Subjects in the Nurses Licensure Examination — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/2025-10%20%20published%20Enhanced%20TOS%20nuse.pdf
Official scope: Nursing Licensure Examination Program, August 2026 — Professional Regulation Commission; Professional Regulatory Board of Nursing — https://www.prc.gov.ph/sites/default/files/exam%20progam%20aug%202026%20nle%20(nursing)%20signed.pdf

Instructional references

Instructional reference: Basic Emergency Care: Approach to the Acutely Ill and Injured — World Health Organization and International Committee of the Red Cross — https://www.who.int/publications-detail-redirect/9789241513081
Instructional reference: Nursing Fundamentals — Open RN; Wisconsin Technical Colleges System — https://wtcs.pressbooks.pub/nursingfundamentals/

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